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3affya Screen Rules

What each screen must have and must not have, drawn from usability research, healthcare and telehealth studies, Arabic and Gulf audience evidence, and marketplace practice. 156 rules with sources, mapped to the 50 screens in the inventory.

Prepared for Yasir Azeem · 19 October 2026 · Version 1
Companion to programmx_aafia-screen-inventory_2026-10.xlsx (column L, Design pointers; Rules sheet). Rules tagged Law are statutes or regulator standards read through law-firm summaries: settle them with Dr Ayman's counsel before relying on them.

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Sources

1. How to read the rules

Each rule has an ID (letter for the area, number within it), the rule in one line, the evidence behind it, and the source. The same IDs appear in the Rule refs column of the screen inventory, so a developer building the search results page can open row 5, read the pointers, and look up S5 or S8 in the Rules sheet or here.

Evidence types

Research a usability study, eyetracking, survey or peer-reviewed paper with numbers.

Guideline a standards body or platform guideline (W3C, WCAG, Apple, Material, HHS, CDC, Stripe docs).

Practitioner a platform's own data or an experienced practitioner's write-up (Zocdoc, Doximity, Growth.Design, CXL). Useful, not proven.

Law a statute or regulator standard, read through a law-firm summary. Verify with counsel.

Three rules of thumb that cover most screens

1. **Say the price, the credential and the next slot before anything else.** Those three are what patients weigh; reviews come last, and we have none yet.
2. **Never leave a gap.** A missing photo, an empty star row, a blank wait, a No results page with nothing to do: each one reads as a broken product. Fill it with the next useful thing.
3. **Arabic is the product, not a translation.** Right-to-left layout, larger type, no letter-spacing, Western digits forced in code, the patient's own time zone and weekend.

A rule marked **Must not** is a prohibition. The rest are things the screen must have.

Where lanes disagreed, the stricter or better-evidenced source won and the other is named in the evidence column. Four gaps were left as defaults rather than rules: religious greetings in copy, Hijri display conventions in Gulf health apps, telemedicine rules for Kuwait, Qatar, Oman and Bahrain, and exact phone-number formats per country.

2. Ten decisions the research forces

These change the plan or need an answer from Dr Ayman or his lawyer. Everything else in this document is a pointer a developer can apply without asking.

1. Cross-border consults from Saudi Arabia Q1 C12

The Saudi telemedicine rules (as summarised by Al Tamimi and the MOH regulation) say a practitioner outside the Kingdom may treat a patient inside it only under supervision of a KSA-based practitioner, that the consultation must include video, and that the patient signs consent before the session. This touches the product framing (first visit vs second opinion vs introduction), the phone visit type, and the consent step. Dr Ayman's lawyer needs to answer this before the Saudi launch messaging is written; the build does not change until then, but the consent step (P9) and the video default should be built so they can be switched.

2. UAE data residency and per-session consent Q2 Q3 T3

UAE health data may not be stored, processed or transferred outside the UAE without health-authority approval, and the DHA telehealth standard wants electronic consent before each consultation and UAE-based servers. Our stack is a single self-hosted Supabase. Options: launch Saudi-first and gate UAE residents from records upload until an approval or a UAE region exists; or get counsel's reading on whether a marketing and introduction platform with the clinical record held by the US doctor falls outside the law. Decide by the 26 Oct checkpoint.

3. Cards: mada will fail, Apple Pay is the Saudi path P12 P13 G6

Stripe on a US or UAE account does not take mada; mada-only debit cards decline. Co-badged cards route via Visa or Mastercard, and Apple Pay and Google Pay work. So the checkout lists Visa, Mastercard, Apple Pay, Google Pay up front, puts the wallets first on phone, and the decline message explains mada. Capture in full at booking; refunds, never voids. No cash on delivery or BNPL.

4. Digits, currencies, weekends, time zones N1 P14 P7 P6

Western digits were decided on 17 Oct; the browser default for ar-SA produces Arabic-Indic digits, so the formatter must pass nu-latn explicitly. KWD, OMR and BHD have three decimals. Weekends differ (Fri-Sat in five countries, Sat-Sun in the UAE). The Gulf has no daylight saving and the US does, so the offset to a US doctor changes twice a year; store UTC and label every displayed time with its zone.

5. Search results: Load more, not pagination; gender and price filters S5 S7 S8

The inventory said pagination, 20 per page. Baymard's testing favours Load more with URL state and scroll restore. The filter list lacked gender, price and next-available; the Riyadh study (n=3,015) makes gender a first-class filter for this audience.

6. No reviews at launch: use experience and verification as the signal C7 C8 U2

Reviews are out of scope for launch. The evidence says that is survivable (55% would still consider a doctor with no reviews; 61% preferred a 4-star doctor with 20+ years over a 5-star with four) if the card shows years in practice, training and a dated verification badge, and labels the doctor New to 3affya rather than showing empty stars. Unclaimed profiles show no score of any kind.

7. Define board certified every time it appears H8 B7 X3

Only 23% of consumers can define it and 78% think it is required to practise. One plain sentence on first appearance, the board named on the profile, and a link to the public verifier. Board eligible is text, never the badge.

8. AI outputs: label, human route, doctor-reviewed state, separate consent T4 T5 T3 Q6 I1 I3

California's AB 3030 is the clearest written standard: AI-generated patient communication must say so and give a human route. Apply it everywhere regardless of state. The summary shows Reviewed by Dr X or Not yet reviewed; the intake chat never outputs a diagnosis or triage level and stops on red flags. Consent to AI processing of records is a separate checkbox from the privacy consent (46% of patients want to be asked).

9. Phone visits are trusted; Saudi may require video C12 Q1 G4

Patients trusted phone more than video for confidentiality (72.5% vs 59.8%), so phone stays a visit type with a best-for line. If counsel confirms the Saudi video rule, the slot picker hides phone for Saudi-located patients. Either way, a one-tap device check before the visit.

10. Reminders moved to January: the evidence says they matter P16 G3

Digital reminders cut no-shows from 21% to 15% across 21 studies. Keep the January date, but budget for 7-day, 1-day and 3-hour reminders with WhatsApp opt-in, because the first bookings will show the cost of not having them.

3. The numbers worth remembering

- 57% of viewing time is above the fold; 74% within two screenfuls (NN/g)
- Hidden navigation: 20%+ less discoverable, 39% slower on desktop (NN/g, n=179)
- Plain-language rewrite: task success 46% to 82% (NN/g, n=50)
- Only 23% can define board certified; 91% rate it important once defined (ABMS)
- Profiles with photos: 60% more engagement (Healthgrades), about 2x views (Doximity, 500k profiles)
- 61% prefer a 4-star doctor with 20+ years over a 5-star with four (Healthgrades)
- Drip pricing lowers trust and rebooking intent (ACM, n=1,000); 40% abandon over extra costs (Baymard)
- Checkout: 11.3 fields on average, 8 are enough; 98% of sites use generic error messages (Baymard)
- Half of new Zocdoc patients book within 72 hours; appointments typically 24 to 72 h out
- Gender preference by specialty is strong in Riyadh (n=3,015)
- 76% of consumers prefer to buy in their own language; 40% will not buy otherwise (CSA, n=8,709)
- WhatsApp: 94% in Saudi, 100% in the UAE (NU-Q)
- Saudi traffic 68% mobile; iOS share 27 to 52% depending on country and source (StatCounter)
- Arabic body text: 60%+ of top Saudi sites set it too small (12 to 16 px)
- Digital reminders cut no-shows 21% to 15% (meta-analysis, 16,000+ patients)
- Patients trusted phone over video for confidentiality, 72.5% vs 59.8% (n=567)
- 96% want results immediately; only 7.5% were more worried (JAMA Netw Open, n=8,139)

- 46% want to be asked before AI touches their records (Univ. of Michigan, n=3,000)
- Only 54 of 100 LLM discharge summaries reached top accuracy; 18 flagged for safety (JAMA Netw Open 2024)
- Symptom checker matched the discharge diagnosis 14% of the time (JMIR 2024 RCT)
- A single 1-star review: 2.3 to 2.6% fewer new patients for 16+ weeks (AJMC, 1.12M appointments)
- Response limits: 0.1 s, 1 s, 10 s (Nielsen); 400 ms Doherty threshold

4. Home and marketing pages

ID	Rule	Evidence	Source
H1 Research	Put the value proposition, the differentiator and the primary CTA in the first screenful, and signal that more follows. Applies: Home, explainer, directory landing	57% of viewing time is above the fold, 74% in the first two screenfuls (NN/g eyetracking 2018).	NN/g, Scrolling and Attention
H2 Research	Write a one-line tagline that says what the site does and for whom; never a generic welcome. **Must not** Applies: Home	Homepage must work as an elevator pitch; vague labels have weak information scent.	NN/g, Homepage Design Principles
H3 Research	Front-load the first two words of every heading, bullet and paragraph with information-carrying words. Applies: All content	F-pattern eyetracking of 232 users: readers catch the first two words of a line far more than the third.	NN/g, F-Shaped Pattern
H4 Research	Use real photos of the actual doctors and team; never stock people or decorative hero figures. **Must not** Applies: Home, about, cards, profiles	Users spent 10% more time on real team portraits than on bios taking 316% more space; stock people are ignored as filler.	NN/g, Photos as Web Content
H5 Research	Show up to 4 top-level links visibly on mobile before hiding the rest; keep visible nav on desktop. Applies: All pages	Hidden nav: discoverability down 20%+, tasks 39% slower on desktop and 15% on mobile (n=179).	NN/g, Hamburger Menus
H6 Research	Show counts (doctors, specialties, bookings) only when they are large enough to impress; small numbers backfire. Applies: Home, directory	A participant judged an article unpopular because it had only 1,000 shares.	NN/g, Social Proof in the UX
H7 Research	Put the three most-trusted credentials (licence, board certification, price) above the fold, ahead of any reviews. Applies: Home, profile	Licence, board certification and insurance/price rated most important when choosing a clinician; reviews rated least.	J Patient Exp 2022, Choosing a Provider
H8 Research	Define US board certified in one plain sentence wherever the badge first appears. Applies: Home, cards, profile, builder	Only 23% of consumers could define board certification; 78% wrongly think it is required to practise; 91% rate it important once defined.	ABMS 2010 Consumer Survey
H9 Research	Show a physical address, the operating entity, and an easy contact route on the home page and footer. Applies: Home, about, contact, footer	Stanford credibility guidelines 2 and 5 (4,500+ participants).	Stanford Web Credibility Guidelines
H10 Guideline	Language switcher in the header, each language in its own script (العربية / English), globe icon, never a flag. **Must not** Applies: All pages	W3C i18n: options in the target language, recognisable icon, keep on-page links even with auto-detect.	W3C, Language selection

ID	Rule	Evidence	Source
H11 Research	Avoid false floors: never end a full-width section with a CTA and whitespace that looks like the page end. **Must not** Applies: Home, explainer	Illusion of completeness (NN/g); cut-off items and rhythm signal continuation (CXL).	NN/g Illusion of Completeness; CXL False Bottom
H12 Guideline	Target LCP of 2.5 s or less at the 75th percentile on mobile; compress doctor photos. Applies: All public pages	Core Web Vitals: good is 2.5 s or less; most Saudi sites miss 3 s because image sets exceed 1,500 KB.	web.dev LCP; Namoun & Alkhodre 2019
H13 Research	Lead with US training plus US board certification as the unique signal. Applies: Home, cards	On Zocdoc, non-US school graduates scored lower (4.63 vs 4.77); Gulf patients already travel to the US for care (USA is Saudi's top destination).	JMIR 2024 Zocdoc reviews; IMTJ GCC medical tourism
H14 Guideline	Show a not-for-emergencies line with the local emergency number for the viewer's country. Applies: Home, intake chat, booking	DHA excludes emergencies from telehealth; HHS: 911 only works in the patient's location.	Kayrouz UAE telehealth; HHS emergency plan
H15 Research	Address legality plainly: a short section on how a US doctor can consult a Gulf patient and what the visit can and cannot do. **Must not** Applies: Home, FAQ, how it works	Saudi survey (n=1,024): 60.5% had used telemedicine but only 41.9% felt the legal aspects were clear.	JMIR Formative 2023, Saudi telemedicine

5. Explainer, FAQ and content pages

ID	Rule	Evidence	Source
X1 Research	Write patient-facing copy at grade 6 reading level; main message in the first paragraph, visible without scrolling. Applies: Explainer, FAQ, emails, summaries, chat	Plain-language rewrite lifted task success 46% to 82% and cut time 22.3 to 9.5 min (n=50); AMA/NIH recommend grade 4 to 6; CDC CCI passing score 90.	NN/g Lower-Literacy; CDC Clear Communication Index
X2 Research	Single main column, static text, linear menus on explanatory pages; no moving text or fly-outs. Applies: Explainer, FAQ	Same NN/g study: two-dimensional layouts and animation hurt lower-literacy readers.	NN/g Lower-Literacy
X3 Research	Make every credential claim verifiable: link to the certifying board or licence registry. Applies: Why US training, profile	Stanford guideline 1: easy third-party verification raises credibility; ABMS Certification Matters is the public lookup.	Stanford guidelines; ABMS Certification Matters
X4 Research	Show a last-reviewed date on medical, process, pricing and legal content. Applies: Explainer, FAQ, legal, profile	Stanford guideline 8: visibly recent content increases credibility.	Stanford Web Credibility Guidelines
X5 Research	Progressive disclosure, two levels at most: core steps first, details behind a labelled control; accordions for many short items, tabs for a few long ones. Applies: How it works, FAQ, profile	Designs beyond two levels usually have low usability.	NN/g Progressive Disclosure; Tabs vs Accordions
X6 Practitioner	Keep the steps list short and highlight the recommended path. Applies: How it works, pricing	Hick-Hyman law: decision time rises with number and complexity of choices.	Laws of UX, Hick's Law
X7 Research	Show a curated set first (top specialties, a few doctors) with see-all, not the whole catalogue. Applies: Directory landing, home	Jam study: 6 options converted 30%, 24 options 3%.	Iyengar & Lepper 2000 via Econsultancy

6. Directory, search and filters

ID	Rule	Evidence	Source
S1 Research	Autocomplete: at most 10 suggestions on desktop, 4 to 8 on mobile; bold the predicted part; arrow keys copy the suggestion into the field. Applies: Search box	80% of sites offer autocomplete, 19% do it well; 58% fail arrow-key copy.	Baymard, Autocomplete Design
S2 Research	Tolerate misspellings and Arabic-to-English transliterations (specialties, names) in autocomplete and results; test symptom words like rash. Applies: Search	69% of sites fail misspellings; 90% of hospital finders failed the word rash and matched surnames instead.	Baymard misspellings; Kyruus via Becker's
S3 Research	Visible search field with a submit button on mobile; persist the query in the field and the filter state in the URL so Back and sharing work. Applies: Search, results	21% lack a mobile submit button; 37% do not persist queries; 90% of load-more sites break Back.	Baymard search benchmark; Baymard/Smashing load-more study
S4 Research	Default sort is diversity-weighted Relevance; never alphabetical or raw popularity. Offer Relevance, Soonest available, Price, Experience. Must not Applies: Results	24% of sites fail the default sort; unrepresentative first pages made users leave; no tester ever chose alphabetical.	Baymard, Default Sort Type; Essential Sort Types
S5 Research	Use Load more, not pagination or infinite scroll: 15 to 30 results on mobile, 25 to 75 on desktop before the button; restore scroll position on Back. Applies: Results	Infinite scroll hid the footer and made users skim; pagination made users browse less; load more performed best.	Baymard via Smashing, Pagination vs Infinite Scroll vs Load More
S6 Research	Zero results must offer: relaxed-filter parent scope, 3 to 5 alternative queries with previews (auto-applied if only one works), popular doctors, and a human contact. Applies: Results, 404	About half of no-results pages raise abandonment; spelling tips alone are not enough.	Baymard, No Results Page
S7 Research	Offer a filter for every attribute shown on the card; include price, language, visit type, gender, next available within 72 h. Applies: Filters	38% of sites lack filters for displayed attributes; 80% of mobile testers tried a price filter; half of new Zocdoc patients book within 72 h.	Baymard, Filters for List Item Info; 5 Essential Filters; Zocdoc
S8 Research	Doctor gender is a first-class filter, suggested for ob/gyn, urology, psychiatry and dermatology. Applies: Filters, card	Riyadh survey (n=3,015): women favoured female physicians for gynaecology, obstetrics, urology and counselling; men preferred male for urology, surgery, orthopaedics.	J Nat Sci Med 2019

ID	Rule	Evidence	Source
S9 Research	Multi-select within a filter type (OR), AND across types; checkboxes not radios; 5 to 10 filter types visible. Applies: Filters	Testers commonly applied 5 to 6 filters, up to 10; 15% of sites block combining values.	Baymard, Multiple Filter Values
S10 Research	Show result counts beside every filter option and on the mobile Apply button (Show 37 doctors); never let a combination land on an empty list. **Must not** Applies: Filters	Counts are one of the highest-impact improvements; they let users avoid zero-result combinations.	Baymard filter UI; NN/g Applying Filters
S11 Research	Desktop: apply filters instantly (under 1 s, dim the list, keep scroll). Mobile: batch behind a sticky Apply in a bottom sheet; sticky filter trigger; top three filters first. Applies: Filters	Four page loads for four filters is too costly on mobile; only 2 to 10% of mobile users open filters vs 38% on desktop.	NN/g Applying Filters; Baymard guideline 1125; Contentsquare
S12 Research	Always show applied filters as removable chips with Clear all; never only a count like Filters (3). **Must not** Applies: Results	28% of sites lack the overview; count-only overviews performed poorly.	Baymard, How to Design Applied Filters
S13 Research	Truncate long value lists at 6 to 10 visible values with a Show more (+N) link; never truncate a single value. **Must not** Applies: Filters (languages, cities)	Under 6 values users assumed the list was complete; 15+ lost overview; inline scroll areas fail on 26% of sites.	Baymard, Truncation Design
S14 Research	List layout (one doctor per row), not a grid of cards, for search results. Applies: Results	Cards de-emphasise ranking and are harder to scan; lists suit homogeneous, comparable items.	NN/g Cards; Baymard product tables
S15 Research	Breadcrumbs: Home > Specialty > City, one line, current page unlinked; parent level only on mobile. Applies: Results, profile	NN/g breadcrumb guidelines.	NN/g, Breadcrumbs
S16 Research	Specialty names bilingual: Arabic first, English in parentheses; language filter names the dialect. Applies: Directory, filters, cards	Gulf users code-switch for medical vocabulary; language-concordant care improved outcomes in 25 of 33 studies.	Diamond et al. JGIM 2019; Saudi code-switching studies
S17 Practitioner	Track fill rate and zeros (searches with no bookable doctor) as the north-star; do not promote a specialty x city combination until it has bookable supply. **Must not** Applies: Directory, ops	Fill rate is the primary marketplace metric; 80% of marketplaces studied went supply-first; Zillow seeded from public records.	Lenny's Newsletter marketplace metrics; a16z; Andrew Chen

7. Doctor card and profile

ID	Rule	Evidence	Source
C1 Research	Every card shows the same attributes in the same order: photo, name, specialty, languages, visit types, next available, all-in price, verification state; say unknown rather than leave a gap. Applies: Card	64% of sites show attributes inconsistently; testers rejected items missing an attribute their neighbours had.	Baymard, List Item Design
C2 Research	Visually separate each attribute (own line or style); whole card clickable, secondary actions behind a divider; key content at the reading-start corner. Applies: Card	40% of sites jumble attributes; unscannable info hurt almost as much as none.	Baymard List Item Design; NN/g Cards
C3 Practitioner	Headshot on every card; missing photo is a blocking profile defect; neutral specialty placeholder for unclaimed, never a broken image. **Must not** Applies: Card, builder	Profiles with photos get 60% more engagement (Healthgrades); about 2x views across 500,000 Doximity profiles.	Healthgrades provider profiles; Doximity 500k profiles
C4 Research	Photo guidance: formal attire with white coat (scrubs plus coat for surgical and ER), plain background, no group or stock photos; a smile is optional. Applies: Builder photo step, admin review	4,062-patient study: white coat rated highest (8.1/10); no significant trust difference between smiling and neutral (n=268).	Petrilli BMJ Open 2018; Univ. Augsburg smile study
C5 Research	Show the all-in consultation price on the card and profile before Book; never a base price with fees added at checkout. **Must not** Applies: Card, profile, checkout	Drip pricing lowered trust and rebooking intent (n=1,000); 21% abandon when total cost is not visible early; 59% say price before care is critical.	ACM/OECD drip pricing; Baymard; AccessOne survey
C6 Practitioner	Show next available slot and Accepting new patients on the card, live from the calendar. Applies: Card, profile	One-third of consumers only view doctors accepting new patients; live availability lifted web booking conversion 30% (self-reported).	Healthgrades; Zocdoc case study
C7 Research	Show years in practice and training as the trust signal while there are no reviews; label New to 3affya instead of empty stars. Applies: Card, profile	61% preferred a 4-star physician with 20+ years over a 5-star with four; hide star averages under 5 ratings; 55% would still consider a provider with no reviews.	Healthgrades; Baymard ratings; Repugen 2024

ID	Rule	Evidence	Source
C8 Research	Verification badge with a plain-language label (Credentials verified, with date), never an icon alone; name the board and link to the public verifier. Must not Applies: Card, profile	Verified-buyer badge raised purchase odds 15%; 56% of consumers do not know if their doctor is certified.	Spiegel Research Center; ABMS survey
C9 Practitioner	Order profile blocks: card data, credentials and training, fees and visit types, availability, bio; education is the second most-weighed factor after reviews. Applies: Profile	Healthgrades consumer data.	Healthgrades provider profiles
C10 Research	Keep the Book CTA visible without scrolling on mobile (sticky). Applies: Profile	Place-order and key options must sit in the first mobile viewport; users do not scroll far enough.	Baymard, Order Review; Guest Checkout
C11 Research	Only authentic scarcity (a real next slot); no countdowns, no only-2-left, no strikethrough prices without a stated basis. Applies: Profile, checkout	11.1% of 11k sites had dark patterns; urgency tactics trigger reactance and damage trust.	Mathur et al. 2019; Growth.Design GoDaddy case
C12 Research	Visit type (video or phone) is an explicit choice with a one-line best-for hint; do not default everyone to video. Note: Saudi rules may require video (Q1). Must not Applies: Profile, slot picker	Patients trusted phone more than video for confidentiality (72.5% vs 59.8%); phone was the top format for 46.5% (n=567).	JMIR Human Factors 2023
C13 Practitioner	Keep name, address and phone consistent with the doctor's own site. Applies: Profile, builder	74.5% of patients check that a provider's details match their website.	Repugen Patient Review Survey 2024

8. Unclaimed profiles and the claim flow

ID	Rule	Evidence	Source
U1 Practitioner	Label unclaimed profiles to patients: Unclaimed, details from public US records, not yet confirmed by the doctor, with source and date; never let them look identical to verified profiles. **Must not** Applies: Unclaimed profile, card	Yelp Unclaimed tag; Google Own this business; Healthgrades auto-profiles carry stale data for years.	Yelp, Google, Healthgrades patterns
U2 Research	No computed score, star row, or quality signal on unclaimed profiles; facts only, with their source. Applies: Unclaimed profile	Avvo was sued within nine days of launch over algorithmic ratings built from public bar data.	Browne v. Avvo (2007)
U3 Practitioner	Rank unclaimed profiles below verified ones so the first page is bookable; give them a Request this doctor path if shown at all. Applies: Results	Zeros (searches with no match) are the liquidity failure to minimise; seeded supply is a bridge, not the product.	a16z network effects; Lenny's Newsletter
U4 Practitioner	Prominent Are you Dr X? Claim this profile, free, plus a visible This is not me / remove link. Applies: Unclaimed profile, claim landing	Zocdoc claim page pattern: preview, editable sections, free, benefits, removal link.	Zocdoc claim page
U5 Practitioner	Claim flow: free, show the already-built profile and what the doctor can edit, verify by NPI plus email OTP, promise you can always edit this. Applies: Claim landing	Zocdoc and Yelp claim patterns; Doximity starts registration from an NPI stub.	Zocdoc claim; Doximity registration
U6 Research	Warn and prepare: claiming invites scrutiny; set up alerts and reply tooling at claim time (for when reviews ship). Applies: Claim landing, builder	After owners claimed Yelp pages, average rating fell over 10% for more than a year.	FIU / Information Systems Research 2026

9. Forms and sign-in

ID	Rule	Evidence	Source
F1 Research	Labels above fields, never placeholder-only; single column; mark optional fields, not required. **Must not** Applies: All forms	Placeholders strain memory and are skipped by screen readers; users ignored asterisks (eyetracking).	NN/g Placeholders; cxpartners eyetracking
F2 Research	Cut fields to the minimum; one Full name field; hide rarely used fields behind links; never re-ask for data already entered. **Must not** Applies: Intake, checkout, builder	Average checkout has 11.3 fields, 8 are enough; 42% typed full name into first-name; WCAG 3.3.7 Redundant Entry.	Baymard checkout fields; W3C SC 3.3.7
F3 Research	Group long forms into chunks of 4 to 6 fields with headings; show all steps with the current one highlighted; allow back and save-and-resume. Applies: Builder, intake, Stripe onboarding	15-field form regrouped into three chunks (NN/g); wizard guidelines; users want to know steps remaining.	NN/g Form White Space; NN/g Wizards
F4 Research	Validate on blur, never on untouched fields; re-check on keystroke once an error exists; clear the error the instant it is fixed. **Must not** Applies: All forms	31% have no inline validation; premature errors made a user feel they broke the site.	Baymard Inline Validation; Smashing live validation
F5 Research	Adaptive error messages that name the specific sub-rule and the fix, next to the field, in text (not colour alone); never the word invalid. **Must not** Applies: All forms	98% of sites use generic errors; some users took 5 minutes or gave up; WCAG 3.3.1.	Baymard Adaptive Errors; NN/g Error Messages; W3C SC 3.3.1
F6 Guideline	OTP: one field, inputmode numeric, autocomplete one-time-code, paste must work; never reveal whether an email exists; rate-limit instead. **Must not** Applies: Sign-in	WCAG 3.3.8: manual transcription fails; split boxes that reject paste are a cited failure.	Cloud Four OTP inputs; W3C SC 3.3.8; Baymard security caveat
F7 Practitioner	Replace the passive check your email screen with a provider-specific inbox link (Open Gmail). Applies: Sign-in	A/B test: +7% email confirmation rate.	Growth.Design, Sniper Link
F8 Research	Right keyboard per field (email, tel, numeric); autocorrect and autocapitalise off on names, emails and codes; inputs rendered at 16 px or larger to stop iOS zoom. Applies: All forms	54% of sites fail keyboard types; iOS Safari auto-zooms inputs under 16 px.	Baymard Touch Keyboards; Strahl iOS auto-zoom
F9 Research	No login wall for browsing; ask for sign-in only at the commitment point (booking, upload). Applies: Sign-in, booking	Login walls are among the most annoying patterns; delayed sign-up gave Duolingo +20% DAU; 24% abandoned carts over forced accounts.	NN/g Login Walls; Baymard Guest Checkout

ID	Rule	Evidence	Source
F10 Guideline	Phone input: GCC country codes first, LTR entry, per-country length validation; collect a backup phone and the patient's country at booking. Applies: Sign-up, booking	Codex phone rules; HHS: pre-agree who calls whom if the visit disconnects, confirm location each visit.	Wikimedia Codex bidi; HHS emergency plan

10. Booking, checkout and payment

ID	Rule	Evidence	Source
P1 Research	Full price and what it includes shown before the final step; the three lines (doctor fee, platform fee, card processing) sum to the total; nothing new at review. **Must not** Applies: Checkout	40% abandon for extra costs; 12% because total was not visible up front; average cart abandonment 70%.	Baymard Cart Abandonment; Order Review
P2 Research	Linear flow: Review step with inline edit links, then a separate Confirmation; review must never look like confirmation. **Must not** Applies: Checkout	11.6% mistook review for confirmation; up to 11% abandonment.	Baymard, Office Depot Order Review
P3 Research	Encapsulate the card fields in a bordered box with one or two security cues inside (lock, Secured by Stripe); 3DS on for all cards. Applies: Checkout	19% abandoned over card-data distrust (n=1,026); 33% of MENA consumers have been fraud victims; 3DS recommended for all Saudi card payments.	Baymard Perceived Security; Checkout.com mada docs
P4 Research	Expiry as MM / YY with input mask; auto-space the card number; keep card details after an error. Applies: Checkout	72% mismatch the printed card format; 34% clear card details on error.	Baymard, Expiration Date Fields
P5 Practitioner	Cancellation and refund policy visible on the checkout step itself, not only in terms. **Must not** Applies: Checkout, profile	Stanford: avoid surprises; common gap on telehealth sites.	Stanford guidelines; Sesame pattern
P6 Guideline	Every slot in the patient's local time with the zone named (3:00 PM Riyadh, GMT+3), doctor's time secondary; store UTC; recompute each render because the Gulf has no DST and the US does. Applies: Slot picker, confirmation, emails	Saudi, Kuwait, Qatar, Bahrain UTC+3; UAE, Oman UTC+4; no DST in the Gulf.	timedate.org Middle East zones; HHS
P7 Guideline	Shade weekends per country: Fri-Sat for Saudi, Kuwait, Qatar, Oman, Bahrain; Sat-Sun for UAE (since 2022); never hard-code Sat-Sun. **Must not** Applies: Slot picker, availability editor	UAE moved to Saturday-Sunday on 1 Jan 2022; the rest keep Sunday-Thursday weeks.	Executive Traveller; Wikipedia Workweek
P8 Practitioner	Collect visit reason, patient age group and for-whom (self or relative) before confirming; let the doctor set lead time (2 h default) and age rules. Applies: Slot picker, checkout	Wrong-fit bookings come from missing reason, age preferences and lead time (Zocdoc); Saudi families often take part in decisions, so ask, do not assume.	Zocdoc wrong-fit bookings; Alfahmi BMC Med Ethics 2022

ID	Rule	Evidence	Source
P9 Guideline	Telehealth informed consent inside the booking flow, before the first visit, recorded with how and when it was given. Applies: Checkout	HHS: consent is typically done before the first appointment and documented.	HHS, Obtaining informed consent
P10 Guideline	Confirmation is the peak and the end: both times, join link, add to calendar, what to prepare (device test, private room, ID), expected lead time, how to change. Applies: Booking confirmation	Peak-end rule; AMA pre-visit checklist (camera, mic, ~384 kbps, private well-lit room, ID).	Laws of UX Peak-End; AMA Telehealth Playbook
P11 Practitioner	Never pre-select paid add-ons; label items Included or Optional; pre-select only what most patients want (reminders on). **Must not** Applies: Checkout, settings	Default bias used against users triggers reactance; opt-out defaults change behaviour about 5x.	Growth.Design GoDaddy; Johnson & Goldstein 2003
P12 Guideline	Accept Visa, Mastercard, Apple Pay and Google Pay and say so up front; expect mada-only debit cards to fail on a non-Saudi Stripe account; Apple Pay is the main Saudi path. Applies: Checkout, pricing, failure state	Stripe UAE/US accounts do not take mada; outside Saudi acquiring, mada cards behave as debit cards; wallets plus mada dominate Saudi e-commerce.	Stripe UAE methods; Checkout.com mada; SAMA
P13 Guideline	Capture in full at booking (no auth-then-capture); refunds not voids. Applies: Checkout, refunds	mada transactions must be captured in full; partial capture and auth-only unsupported.	Checkout.com mada docs
P14 Guideline	Price in the patient's currency (SAR, AED, QAR, KWD, OMR, BHD); KWD, OMR and BHD use three decimals, round to the nearest 10 fils (verify Stripe rounding). Applies: Pricing, checkout, receipts	Local currency improves conversion and authorisation rates (Stripe).	Stripe currencies
P15 Law	Keep a Good Faith Estimate style breakdown in the portal after payment. Applies: Receipts, checkout	US No Surprises Act: self-pay patients get a written estimate when scheduling 3+ business days ahead.	CMS, Understanding costs in advance
P16 Research	Reminders (when they ship in January): 7 days, 1 day, and about 3 hours before; opt-in WhatsApp where allowed. Applies: Reminders, settings	Digital reminders cut no-shows 21% to 15% (21 studies, 16,000+ patients); WhatsApp used by 94% in Saudi, 100% in UAE (NU-Q).	BMJ Open 2016 meta-analysis; Zocdoc cadence; NU-Q

11. Patient portal, records and AI summary

ID	Rule	Evidence	Source
T1 Guideline	View and download are the two primary portal actions; phone-first layout. Applies: Bookings, summary	90% of portal users view results, 70% notes, 32% download; 51% use an app; 57% of adults use a portal.	ONC Data Brief 69 (2022)
T2 Research	Release the summary to the patient immediately with a worry-reducing framing line; add context to abnormal flags. Applies: Summary viewer	96% prefer immediate access (n=8,139); 7.5% were more worried, nearly half less; abnormal-result viewers 3x as likely to worry.	JAMA Netw Open 2023 via UT Southwestern
T3 Research	Separate, explicit consent before any uploaded record is processed by AI, distinct from the privacy consent; say where data is stored and processed. Applies: Records upload	46% want permission asked before AI is used on their records, 38% notification (n=3,000, Jan 2026); UAE law restricts health data leaving the country.	Univ. of Michigan AI consent survey; UAE ICT Health Law
T4 Law	Label every AI summary as AI-generated at the top with a one-tap route to a human; show Reviewed by Dr X or Not yet reviewed. Applies: Summary viewer	California AB 3030 requires AI disclosure and a human route; only 54 of 100 LLM summaries reached top accuracy, 18 flagged for safety risk.	Duane Morris on AB 3030; JAMA Netw Open 2024
T5 Guideline	Not a diagnosis, does not replace the consultation: in the body of the summary, not only a footer. Must not Applies: Summary viewer	Disclaimer rates in AI health answers fell from 26% to 1% (2022 to 2025); FTC: no AI exemption.	eMarketer on MIT Tech Review; FTC Operation AI Comply
T6 Guideline	Every statement links to its source page; explain every number (what it means, normal range); show what we know and what we do not; a something missing? link. Must not Applies: Summary viewer	CDC CCI items 15 to 17; FDA CDS: basis must be reviewable; 96% of OpenNotes patients understood notes, 6% found something missing.	CDC Clear Communication Index; OpenNotes JGIM
T7 Research	Who can see this panel on every record and share; share expiry visible; revoke in one tap. Applies: Sharing and consent	Over 12% withheld information from providers over privacy fears; only about 25% very confident in record privacy.	JMIR privacy survey via Becker's
T8 Research	Delete my uploads and Delete my account controls that state what deletion covers and what legal retention keeps. Applies: Settings, records	37% think AI will make record security worse; UAE mandates 25-year retention for provider records.	Pew 2023; Al Tamimi UAE ICT Health Law
T9 Law	Notice of Privacy Practices posted prominently, plain language, effective date, complaints contact; acknowledgment captured at first use. Applies: Legal pages, portal	HHS: prominently post the notice on any site about services; deliver with the first electronic service.	HHS, Notice of Privacy Practices

ID	Rule	Evidence	Source
T10 Research	Upload designed for phone photos of paper reports first; accepted formats, size limit and per-file success state visible. Applies: Records upload	Portal upload adoption is slow when friction is high (53 uploaders in four years at one practice).	Applied Clinical Informatics 2019
T11 Research	Show the work during processing: a live step list (Reading page 3 of 12, Extracting diagnoses, Writing summary); no fake percentage; over 10 s show an estimate and a cancel. Applies: Records processing	Labour illusion: users preferred a slower site that showed its work; 86% of users preferred a progress bar; never let a bar move backwards.	Buell & Norton 2011; UX Tigers progress indicators
T12 Research	Confidentiality statement in plain Arabic at booking and in the consult lobby; never push health details through marketing channels. **Must not** Applies: Booking, lobby, settings	Worry about online monitoring rose from 34% to 44% across seven Arab countries (n=7,303).	NU-Q Media Use 2019

12. Professional portal and profile builder

ID	Rule	Evidence	Source
B1 Practitioner	Start from the pre-seeded stub (Find your profile), never a blank form. **Must not** Applies: Builder, claim	Doximity registration starts by claiming an NPI stub; about 70% of US doctors are on it.	Doximity registration
B2 Research	Required fields are only what admin will verify (NPI, licence, board, specialty, photo); everything else is complete later. Applies: Builder	Practices spend about USD 2.76B a year updating directory data; 40%+ never correct their listings.	AHA on CAQH; Healthgrades
B3 Research	Endowed progress: start the checklist partly filled from registry data and show steps remaining. Applies: Builder, Stripe onboarding	12-stamp card with 2 free stamps completed in median 10 days vs 15; Zeigarnik effect.	Kivetz et al. via Columbia Business School
B4 Research	Live preview card that renders exactly as the public card; show the benefit at the step (photos get about 2x views). Applies: Builder	NN/g heuristics 1 and 6; Doximity 500k-profile analysis.	NN/g 10 Heuristics; Doximity
B5 Research	Deliver value before asking for effort: show the claimed card and benefits before Stripe onboarding. Applies: Builder, payouts	Fogg B=MAP; reciprocity.	Fogg Behavior Model; Growth.Design psychology
B6 Practitioner	Structured fields, not free text: years in practice, board and status, training, languages with dialect, Accepting new patients (default off until availability is set). Applies: Builder	NCQA primary-source verification needs board, licence and training as separate items; one-third of patients filter on accepting.	Verifiable on NCQA; Healthgrades
B7 Research	Board eligible is a distinct text label, never the badge; reject board certified claims without a named ABMS/AOA board. **Must not** Applies: Builder, admin review	78% of consumers conflate licensure with certification; FTC treats unsubstantiated claims as deceptive.	ABMS survey; FTC
B8 Research	Membership page: three tiers, at most four attributes differing between adjacent tiers, features carry upward, all-in price and billing period up front, static comparison table. Applies: For professionals, membership	Good-Better-Best (HBR 2018); NN/g comparison tables; drip pricing harms trust.	HBR Good-Better-Best; NN/g Comparison Tables
B9 Practitioner	Sell membership on patient-side outcomes with numbers: live availability (half of new patients book within 72 h), photos (2x views), fast response. Applies: For professionals	Zocdoc patient access data; Doximity.	Zocdoc patient access

ID	Rule	Evidence	Source
B10 Research	Prefer undo over confirmation for routine actions; confirmation dialogs only for irreversible actions, buttons that name the outcome (Cancel booking / Keep booking). Applies: Portals, admin	Frequent confirmations train click-through.	NN/g Confirmation Dialogs

13. Admin review and verification

ID	Rule	Evidence	Source
A1 Practitioner	Verify licence, board certification and sanctions against primary sources (state board, ABMS, FSMB/NPDB) before Verified shows; store source, date and reviewer. Applies: Credential review	NCQA primary-source verification elements.	Verifiable on NCQA PSV
A2 Practitioner	Verified expires: schedule re-verification (licence at expiry, board per cycle) and show verified on [date] to patients. Applies: Credential review, listings	NCQA recredentialing every 2 to 3 years; Stanford visible recency.	Verifiable on NCQA; Stanford
A3 Research	Queue removal requests and (later) 1-star reviews for human moderation with evidence rules; providers can never delete reviews. **Must not** Applies: Removal requests	A single 1-star review was followed by a 2.3 to 2.6% drop in new-patient volume for 16+ weeks (1.12M appointments).	AJMC 2023; Zocdoc moderation
A4 Law	Label every paid placement Sponsored or Paid placement at the reading-start corner of each card, with shading or a border; never Featured by or a logo alone; disclose if tier affects ranking. **Must not** Applies: Featured banner, results	FTC native advertising guide and 2013 search-engine letter.	FTC Native Advertising Guide
A5 Research	Admin tables: progressive disclosure, rarely used options one level down, frequency logged. Applies: All admin	NN/g progressive disclosure.	NN/g Progressive Disclosure

14. Intake chat

ID	Rule	Evidence	Source
I1 Research	First message: this is an AI assistant, not a doctor; not a diagnosis; the 3 to 5 things it can do; emergency number for the viewer's country; persistent Talk to a human button. Applies: Intake chat	Users adjusted once they knew it was a bot; unclear scope left users stuck; AB 3030 human route; 60% uncomfortable with AI in their care.	NN/g Chatbots; Duane Morris AB 3030; Pew 2023
I2 Research	Tappable quick replies for common answers alongside free text; tolerate typos; keep context across turns; allow going back; say I did not understand instead of guessing. Applies: Intake chat	Domino's re-asked an address; No dead-ended a Kia bot; users preferred honest failure.	NN/g, The User Experience of Chatbots
I3 Research	Never output a diagnosis or triage level; collect structured history and suggest specialties only; hand off to the doctor. Must not Applies: Intake chat	Symptom checker RCT (n=437): top suggestion matched discharge diagnosis in 14%; over-triage 53%.	JMIR 2024 Ada vs Symptoma
I4 Research	Hard-coded red-flag detection (chest pain, stroke signs, pregnancy bleeding, self-harm) interrupts with emergency guidance and ends intake. Applies: Intake chat	Symptom checkers miss life-threatening cases; HHS requires an emergency plan.	JMIR 2024; HHS emergency plan
I5 Research	Do not collect sensitive data (date of birth, card, full address) in chat; hand off to a form. Must not Applies: Intake chat	Participants hesitated to share birthdays, addresses and card numbers in chat.	NN/g Chatbots

15. Transactional email

ID	Rule	Evidence	Source
E1 Research	Subject states the content (Your visit with Dr X is confirmed, 14 Nov, 6 pm Riyadh); preheader carries real info, not view in browser. Applies: All emails	NN/g diary study: uninformative subjects and wasted preheaders drew the harshest comments.	NN/g, State of Transactional Email
E2 Research	Answer every likely question in the body (what, when in both zones, how to join, how to change, what to prepare); no ads; works on a small screen. Applies: Booking, OTP, receipts	Users annoyed by emails that require extra work or carry sales pitches.	NN/g, State of Transactional Email
E3 Research	Receipt mirrors the review step: total most prominent, the three lines, last 4 digits, both times, change and cancel links. Applies: Receipt, refund	Baymard order-review information list.	Baymard, Office Depot Order Review
E4 Guideline	RTL email: dir=rtl on the table cells, Arabic at 16 px or larger, numbers and codes isolated LTR; test in Gmail and Outlook. Applies: All emails	W3C inline bidi; Apple +2 pt guidance.	W3C inline bidi markup

16. Loading, empty and error states

ID	Rule	Evidence	Source
L1 Research	Response limits: under 1 s no indicator; 1 to 10 s a content-shaped skeleton (onset after about 1 s) or a module spinner; over 10 s a progress bar with estimate and cancel. Applies: All screens	Nielsen's three limits; frame-only skeletons make users think the page is broken.	NN/g Response Times; Skeleton Screens 101
L2 Practitioner	Respond to every tap within 400 ms with at least an optimistic change (filters, slot holds, chat). Applies: Filters, availability editor, chat	Doherty threshold (IBM 1982).	Laws of UX, Doherty Threshold
L3 Research	Every empty state says what the container is, why it is empty, and gives one direct action; never show No records while content is still loading. Must not Applies: Bookings, records, inbox, admin queues	NN/g empty-state guidelines.	NN/g, Designing Empty States
L4 Research	Errors: exact problem next to the element, the fix, input preserved, no blame, no humour, no codes by default. Applies: All screens	NN/g error-message guidelines; WCAG 3.3.1.	NN/g Error-Message Guidelines

17. Arabic, RTL and typography

ID	Rule	Evidence	Source
R1 Guideline	dir=rtl and lang=ar on html for the Arabic locale; the whole layout, nav order and alignment flip, not just the text. Applies: All screens	Apple HIG and Material bidirectionality.	Apple HIG, Right to left
R2 Guideline	Mirror: back and next, chevrons, steppers, progress bars (0% on the right), pagination, carousels, text-alignment icons. Do not mirror: media controls, clocks, search magnifier, pencil, checkmarks, logos, card-scheme marks, photos. **Must not** Applies: All screens, brand page	Apple HIG, Material m1, Wikimedia Codex, Microsoft mirroring guidance agree.	Apple HIG RTL; Material m1 Bidirectionality; Codex
R3 Guideline	Never reverse digits: phone numbers, card numbers, OTP codes, booking references stay LTR, left-aligned, wrapped in dir=ltr; use bdi or dir=auto for database strings (doctor and hospital names). **Must not** Applies: Forms, confirmation, emails, cards	W3C: wrap every opposite-direction phrase; Codex: phone numbers never mirrored.	W3C inline bidi; Codex
R4 Guideline	Align 3+ line paragraphs by their own language (English bio inside an Arabic page stays left-aligned); 1 to 2 line labels follow the page direction; list items share one alignment even when some are Latin. Applies: Profile bio, credentials, legal	Apple HIG RTL.	Apple HIG, Right to left
R5 Research	Arabic body 16 px floor on mobile, 18 px preferred; about +2 pt when Arabic sits beside uppercase Latin; line height 1.5 or more. Applies: All screens, brand page	Apple +2 pt; over 60% of 73 Saudi sites set paragraphs at 12 to 16 px, judged too small; Arabic extenders need extra leading (W3C alreq).	Apple HIG; Namoun & Alkhodre 2019; W3C alreq
R6 Guideline	letter-spacing: 0 on [lang=ar]; font-synthesis: none; real Readex Pro weights; no italics for Arabic (emphasise with weight or colour); no text-align: justify. Applies: Brand page, all screens	Spacing joined letters creates undesirable results (W3C alreq); browsers lack kashida justification; obliques make letters look alike.	W3C alreq; thoughtbot obliques
R7 Guideline	Never style or border individual Arabic letters (search highlighting, input masks); highlight whole words. **Must not** Applies: Search, forms	Joined letters must not be separated by borders (W3C alreq).	W3C alreq

ID	Rule	Evidence	Source
R8 `Practitioner`	Budget 20 to 30% extra width for Arabic strings; never put text inside images; one typeface family (Readex Pro) for both scripts. **Must not** Applies: Nav, buttons, tabs, badges	A 10-letter English word may become a 2 to 3 word Arabic phrase; 55% of Saudi sites mixed fonts, treated as a usability problem.	freeCodeCamp Arabic UX; Namoun & Alkhodre
R9 `Research`	In Arabic, the reading-start corner is top-right: CTA, doctor name, price and next slot go there (flipped F). Applies: Home, results, card	RTL readers follow a flipped F pattern; KSU eyetracking: Arabic users scanned results right to left, attention on the top two.	NN/g F-Pattern; Al-Rashed 2010 KSU
R10 `Research`	Usability-test mirrored controls (sliders, steppers, calendars) with Gulf users; a native Arabic speaker proofreads rendered screens, not strings. Applies: Prototype review, QA	Survey of 234 bidi speakers: preferences often did not match vendor guidelines; broken joins and reversed numbers only show when rendered.	Goldenberg & Tractinsky 2021; freeCodeCamp
R11 `Research`	Charts: numerals LTR, axis order mirrored by hand; do not trust library defaults. **Must not** Applies: Admin dashboards, summary timeline	11 Arabic-speaking practitioners: RTL support in chart tools is fragmented.	Alebri et al. 2026

18. Numbers, dates, time and currency

ID	Rule	Evidence	Source
N1 Guideline	Western digits everywhere, forced in code: Intl.NumberFormat('ar-SA-u-nu-latn') and ar-AE-u-nu-latn; CLDR defaults ar-SA to Arabic-Indic digits. Applies: All screens, config	Saudi is singled out as preferring Western numerals, but CLDR's default for ar-SA is native digits.	Wikipedia Eastern Arabic numerals; CLDR-users 2019
N2 Guideline	Numbers run LTR inside Arabic sentences; currency sign is part of the number; percent sign left of the number with no space; eastern separators 1,234.50. Applies: Prices, receipts	W3C alreq 6.1.	W3C alreq
N3 Guideline	Gregorian as the primary calendar; Hijri optional secondary (useful around Ramadan and Eid). Applies: Slot picker, confirmations, receipts	Saudi Cabinet approved Gregorian for official dealings (31 Oct 2023); banks already used it.	Gulf News; Arab News 2023
N4 Guideline	State the format and unit beside every input (DD-MM-YYYY, SAR); default date fields to today. Applies: Forms, checkout	Dubai Government Design System U.05.	Dubai Design System usability

19. Gulf audience: trust, payments, mobile

ID	Rule	Evidence	Source
G1 Research	Arabic default, localised to full depth (bios, consent, receipts, support), not just the chrome. Applies: All screens	76% prefer to buy in their own language; 40% will not buy in another language (8,709 consumers, 29 countries).	CSA Research 2020
G2 Research	Real human photos, conservative dress, both genders, some group imagery; nothing that reads as non-local stock. Applies: Home, about, profiles	Of 73 top Saudi sites, 90% include humans, 81% conservative imagery, 68% both genders.	Namoun & Alkhodre 2019
G3 Research	WhatsApp click-to-chat for support and (opt-in) reminders; but never health details through it without consent. **Must not** Applies: Contact, confirmation, settings	WhatsApp used by 94% in Saudi, 100% in UAE; 32% name it the most private platform.	NU-Q 2015 and 2019
G4 Research	One-tap pre-call device check and an installation-free join; keep a phone fallback. Applies: Confirmation, lobby	Saudi n=916: 92.8% preferred virtual, 20.5% had technical difficulties; n=385: about half found video visits complex.	MDPI Healthcare 2024; Frontiers 2024
G5 Research	Design phone-first and test iOS and Android equally; Saudi traffic is 68% mobile; iOS share varies 27 to 52% by country and source. Applies: All screens	StatCounter Sept 2026: Saudi iOS 51.6%, UAE iOS 26.7%, Kuwait Android 51.6%.	StatCounter
G6 Practitioner	No cash on delivery or BNPL at launch; cards plus wallets only. Applies: Checkout	COD fell to about 10% in Saudi, UAE, Kuwait; BNPL about 4%; 85% of Saudi retail payments electronic in 2025.	Gulf Business/Checkout.com 2024; SAMA
G7 Practitioner	Neutral respectful register (أهلاً بك); no scripted religious phrases in system copy; test with users. (No reliable source found; treat as a default.) Applies: All Arabic copy	Gap in the research; practitioner default.	None

20. Accessibility (WCAG 2.2)

ID	Rule	Evidence	Source
W1 Guideline	Text contrast 4.5:1 or more (3:1 for 24 px+); check gold on sand and white on lagoon. Applies: All screens, brand page	WCAG 2.2 SC 1.4.3.	W3C SC 1.4.3
W2 Guideline	Pointer targets at least 24x24 px; 48 px for primary mobile actions, slot buttons and chips. Applies: Slot picker, chips, icon buttons, chat quick replies	WCAG 2.2 SC 2.5.8; Fitts's law; Digital.gov 48 px.	W3C SC 2.5.8
W3 Guideline	Focus visible and never hidden by sticky headers, sticky CTAs or banners (scroll-padding). **Must not** Applies: All screens	WCAG 2.2 SC 2.4.7 and 2.4.11.	W3C SC 2.4.11
W4 Guideline	Never block paste or autofill on sign-in fields; label fields so password managers can fill them. **Must not** Applies: Sign-in	WCAG 2.2 SC 3.3.8.	W3C SC 3.3.8

21. Regulatory items to settle with counsel

Counsel items

These are not screen rules. They are the regulatory findings that shape scope, consent and data location. Each is a law-firm summary, not regulator text; the MOH PDF is dated. Put them on the 26 Oct checkpoint agenda and send to Dr Ayman's lawyer with the legal-pages brief.

ID	Rule	Evidence	Source
Q1 Law	Saudi: a practitioner outside the Kingdom may treat a patient inside it only under supervision of a KSA-based practitioner; teleconsultations must include video; the patient signs consent before the session; identity verified. Decide the product framing (second opinion, Saudi partner physician) with counsel. Applies: Scope, consent, visit types, legal pages	Al Tamimi summary of Saudi telemedicine licensing; MOH telehealth regulations PDF (valid till March 2022, check for successor).	Al Tamimi KSA telemedicine; Saudi MOH regulations
Q2 Law	UAE: the patient's location sets the regulator; Abu Dhabi allows telecounselling from outside the UAE but telediagnosis only between licensed facilities; DHA Telehealth Standard requires electronic consent before each consultation, Arabic and English interfaces, UAE-based servers, no remote controlled-substance prescribing. Applies: Scope, consent per session, architecture	Kayrouz & Associates; Al Tamimi UAE; Neelim 2026.	Kayrouz, UAE telehealth licensing and data
Q3 Law	UAE health data may not be stored, processed or transferred outside the UAE without health-authority approval (Federal Law 2 of 2019, Art. 13; fines AED 500k to 700k; 25-year retention). Plan data residency or an approval route before UAE launch. Applies: Architecture, privacy, records upload	Al Tamimi; Clifford Chance 2025.	Al Tamimi, UAE ICT Health Law
Q4 Law	Saudi PDPL cross-border transfer needs adequacy, standard clauses or a derogation; confirm the health-data position with counsel. Applies: Architecture, privacy	Clifford Chance 2025 (does not address health data specifically).	Clifford Chance, Data transfers UAE and KSA

ID	Rule	Evidence	Source
Q5 `Practitioner`	Kuwait, Qatar, Oman, Bahrain: no cross-border telemedicine rules found in this research (regulators: Kuwait MOH, QCHP, Oman MOH, NHRA). Treat as a gap for counsel. Applies: Scope	Neelim 2026 covers only UAE and KSA.	Neelim, GCC telemedicine licensing 2026
Q6 `Law`	California AB 3030 (from 1 Jan 2025): AI-generated patient communications about clinical information must disclose AI generation and how to reach a human, unless a licensed provider reviewed them. Apply to the summary and the intake chat regardless of state. Applies: Summary viewer, intake chat	Duane Morris alert.	Duane Morris on AB 3030

22. Pointers screen by screen

For each screen: the layout call (column N of the inventory: the stack top to bottom on a phone, what sits above the fold, and the hook that keeps the person on the page), then the pointers from column L with the rule IDs behind them. Layout calls are judgements made with the whole product in view: one shared frame, one card, portals with bottom tabs, admin table-first.

Public

1. Home

Land, search, trust · M2 · task 1.6, 2.1

Layout: Lagoon hero (approved board) with headline, one-line sub, search box with specialty autocomplete, four specialty chips. Then a trust strip (Licence verified, US board certified with tooltip, visit from USD X). Then two verified doctor cards, three steps, FAQ teaser, footer with address. Above the fold: headline, search, chips. Hook: the search box returns live suggestions as you type.

1. First screenful: tagline that names the buyer (US-trained, Arabic-speaking doctors for you), search box, one CTA. In Arabic the CTA and headline sit top-right. `[H1 H2 R9]`
2. Trust strip above any counts: Licence verified, US board certified (one-sentence definition on first appearance), price from X. Lead with US training. `[H7 H8 H13]`
3. Real doctor photos only (no stock); conservative dress, both genders. Show counts only if large; otherwise show two verified doctors instead of a number. `[H4 G2 H6]`
4. Up to 4 visible nav links on mobile; language switch in the header as العربية / English with a globe, no flags. `[H5 H10]`
5. One line: not for emergencies, with the viewer's local emergency number; a short how a US doctor can see you section or link (41.9% of Saudi users unclear on legality). `[H14 H15]`
6. No false floor after the hero; LCP 2.5 s on mobile (compress photos, water effect must not block paint); content-shaped skeleton for the doctor strip. `[H11 H12 L1]`

2. Why US training

Explain the value of US-trained doctors · M2 · task 1.6

Layout: Single column: title, one-paragraph answer, then three short sections (what US training means, how we verify, what it costs you), each with a link to the verifier. Sidebar on desktop holds the three-credential strip. Above the fold: the one-paragraph answer. Hook: a Check a doctor link into the directory.

1. Grade-6 copy, single column, main message in the first paragraph; front-load headings. `[X1 X2 H3]`
2. Link every credential claim to ABMS Certification Matters or the state board; define board certified; last-reviewed date at the bottom. `[X3 H8 X4]`
3. Use the medical-tourism fact (USA is Saudi's top destination) as the bridge; explain how the visit works legally in one paragraph. `[H13 H15]`

3. How it works

Patient journey in 3 steps · M2 · task 1.6

Layout: Three numbered steps as stacked cards (find, book, talk), each with one line and a small illustration; fees box under step 2; visit types under step 3; what to prepare as an accordion; CTA to search at the end and sticky on phone. Above the fold: step 1 and 2. Hook: fee example with the three lines filled in.

1. Three steps, two levels of detail at most, recommended path highlighted; make clear no account is needed to browse. `[X5 X6 F9]`
2. Explain fees as one all-in number plus what it includes; cancellation terms here too; video vs phone with a best-for line. `[C5 P5 C12]`
3. What to prepare for a visit (device test, private room, ID) and the not-for-emergencies line. `[H14 P10]`

4. Specialty directory

Browse 104 groups · M2 · task 2.1

Layout: Six curated specialties as large tiles, then See all 104 expands an alphabetical two-column list grouped by body system, Arabic first with English small. Count badge on each. Above the fold: the six tiles and the search box. Hook: tiles show one doctor photo each.

1. Curated top specialties first with see all 104; names Arabic first with English in parentheses. `[X7 S16]`
2. Counts per group (verified only); hide groups with zero bookable doctors from promotion rather than showing 0. `[S10 S17 H6]`
3. Grid is fine here (heterogeneous browse); in Arabic the first item is top-right. `[S14 R9]`

5. Search results

Find a doctor by filters · M2 · task 2.1

Layout: Sticky top bar: query, Filters button with count, sort. Applied-filter chips row. One doctor per row: photo left (right in RTL), name and specialty, languages, next available, all-in price, Book. Load more after 20 on phone. Desktop: filters in a left column, list right. Above the fold: first two rows. Hook: next available slot on every row.

1. One doctor per row, same attribute set in the same order; key data at the reading-start corner. `[S14 C1 C2 R9]`
2. Replace pagination with Load more (15 to 30 mobile, 25 to 75 desktop); filter state in the URL; restore scroll on Back. `[S5 S3]`
3. Default sort Relevance, verified above unclaimed; any paid placement carries a Sponsored label on the card itself. `[S4 U3 A4]`
4. Add gender, price and next-available-72h to the filters; counts per option; mobile bottom sheet with Show N doctors; applied chips with Clear all; truncate long lists at 6 to 10. `[S7 S8 S9 S10 S11 S12 S13]`
5. Autocomplete capped at 10/8; transliteration and typo tolerance; symptom words map to specialties. `[S1 S2]`
6. Zero results: relaxed scope, 3 to 5 alternatives, popular doctors, WhatsApp/contact; skeleton shaped like rows. `[S6 L1 L3]`

6. Doctor profile: unclaimed

Public registry listing · M2 · task 2.2

Layout: Grey Unclaimed banner with source and date at the top. Placeholder photo, name, specialty, city. Facts table (school, year, group, hospital). Then a teal panel: Are you Dr X? Claim this profile, free. Then Find a verified doctor in this specialty. Above the fold: banner, name, claim panel. Hook: claim panel shows the finished card they would get.

1. Banner: Unclaimed, from public US records (source, date), not confirmed by the doctor. Facts only, no score or stars; neutral specialty placeholder photo. `[U1 U2 C3]`
2. Are you Dr X? Claim this profile, free (preview of what they can edit) plus This is not me / remove link. `[U4 U5]`
3. No Book, no fee, no availability (as planned); offer Find a verified doctor in this specialty so the patient is not stranded. `[U3 S6]`

7. Doctor profile: verified

Book a first visit · M2 · task 2.2

Layout: Card front as the header (photo, names, specialty, city, verified badge with date). Then fee and next three slots with a sticky Book bar on phone. Then credentials and training, languages and visit types, bio, availability preview. Desktop: card and booking box pinned right, content left. Above the fold: card, fee, next slots. Hook: tap the card to flip to the back.

1. Block order: card, credentials (board named, verified on date, link to verifier), fees and visit types, availability, bio. Define board certified on first use. `[C9 C8 H8 X3]`
2. All-in price and next available above the fold; sticky Book on mobile; Accepting new patients state. `[C5 C6 C10]`
3. No reviews at launch: show years in practice and training as the trust signal and a New to 3affya label; no scarcity tricks. `[C7 C11]`
4. Video vs phone with a best-for line (see Q1 for Saudi); cancellation terms near Book; English bio stays left-aligned inside the Arabic page. `[C12 P5 R4]`

8. Intake chat

Suggest a specialty, prefill introduction request · M3 · task 3.12

Layout: Full-height sheet over the page on phone, right-side panel on desktop. Bot message with scope, emergency line and Talk to a human pinned at the top. Quick-reply chips under each question; free-text box at the bottom. Result: three specialty cards each with two doctors. Above the fold: first question and chips. Hook: the result cards link straight to filtered search.

1. Opening message: AI assistant, not a doctor, not a diagnosis, what it can do, local emergency number, Talk to a human always visible. `[I1 H14 Q6]`
2. Quick replies for concern, age group, language, visit type; no date of birth or card in chat; back and I did not understand supported. `[I2 I5]`
3. Output is 1 to 3 specialties with doctors, never a diagnosis or triage level; red-flag keywords end intake with emergency guidance. `[I3 I4]`
4. Consent line before the first question; typing indicator under 1 s, step text if a reply takes longer. `[T3 L1]`

9. Claim invite landing

Doctor opens invite link · M2 · task 2.3

Layout: Centred single column: the pre-built card at the top (name, specialty from registry), one sentence of what claiming gives, email field pre-filled, Send code button, then the six-digit field. Expired or used token shows the same card with Request a new link. Above the fold: card and email field. Hook: the card is already theirs.

1. Show the pre-built profile first (name, specialty from registry), then OTP; promise you can always edit this; checklist starts partly filled. `[U5 B1 B3]`
2. OTP single field, numeric keyboard, paste works; Open Gmail link; rate-limit, do not reveal whether the email exists. `[F6 F7 W4]`
3. Visible This is not me; one line that claiming means patients will see you, so keep availability current. `[U4 U6]`

10. This is me / remove me

Doctor asks for removal or claims · M2 · task 2.2, 2.8

Layout: Short centred form: two radio choices (This is me, Remove me) that change the copy, then name, NPI, email, reason. Submitted state: what happens and the 48-hour promise. Above the fold: the whole form. Hook: none needed; keep it under one screen.

1. Four fields, labels above, specific errors; NPI field LTR numeric. `[F1 F2 F5]`
2. Submitted state names the 48-hour SLA and what happens next (410 within 48 hours). `[U4 A3]`

11. AI tools (coming soon)

Tease agents 2 to 11 · M2 · task 1.6

Layout: Title, one line of intent, then a two-column list of ten agents with name and one line each, muted style. One CTA: Tell me when these launch (email). Above the fold: title and first four. Hook: the notify field.

1. One plain line per agent, front-loaded; no pricing. `[X1 H3]`
2. Say every AI output will be labelled and reviewed; not a diagnosis. `[T4 T5]`

12. For professionals

Explain membership and claim flow · M2 · task 1.6

Layout: Hero line for doctors, then the practice card shown large as the reward, then three steps (claim, verify, go live), then a three-column tier table (stacked on phone, middle tier highlighted), then FAQ for doctors. Above the fold: hero and card. Hook: Find your profile search box in the hero.

1. Three tiers, at most four differing attributes, all-in price and billing period, static comparison table (prices placeholder until 7 Nov). `[B8 C5]`
2. Sell on numbers: live availability (half of new patients book within 72 h), photo (2x views); show the practice card as the reward. `[B9 B4]`
3. How claiming works: free, from your registry profile, only verifiable fields required; note that patients will scrutinise a claimed profile. `[U5 U6 B2]`

13. FAQ

Common questions · M2 · task 1.6

Layout: Search-within-FAQ field, then grouped accordions (For patients, Fees and refunds, Records and AI, For doctors). Each answer under six lines. Last-reviewed date at the bottom. Above the fold: search field and first group. Hook: the search field.

1. Grade-6 answers, accordion, two levels; include How can a US doctor see me? and What this visit cannot do. [X1 X5 H15]
2. Fees, refunds and how records are processed (AI consent, where stored) answered here too. [C5 P5 T3]
3. Last-reviewed date. [X4]

14. About

Who 3affya is · M2 · task 1.6

Layout: Single column: who 3affya is in two paragraphs, Dr Ayman photo and one-line bio, how verification works, entity and address, contact. Above the fold: the two paragraphs and photo. Hook: the verification explainer link.

1. Operating entity, address, contact; real team and doctor photos. [H9 H4 G2]
2. Last-reviewed date; link to how verification works. [X4 X3]

15. Contact

Reach 3affya · M2 · task 1.6

Layout: Two stacked choices: WhatsApp button first, then the three-field email form. Address and hours under it. Above the fold: both choices. Hook: WhatsApp opens with a pre-filled greeting.

1. WhatsApp click-to-chat first, then email form; address shown. [G3 H9]
2. Labels above, specific errors; sent state says when to expect a reply. [F1 F5 L4]

16. Legal pages (4)

Privacy, terms, fees, refunds · M4 · task 1.6, 4.3

Layout: One template: title, effective date, a four-line plain summary box at the top, then numbered sections with a sticky in-page contents on desktop. Above the fold: the summary box. Hook: the summary box.

1. Privacy page doubles as the Notice of Privacy Practices: prominent, plain language, effective date, complaints contact. [T9 X4]
2. Counsel must settle: cross-border consult framing (Saudi supervision and video rule), UAE data residency, retention and deletion wording. [Q1 Q2 Q3 Q4 T8]
3. Fees and refunds page: the three lines, capture in full, refund not void, written estimate kept in the portal. [P5 P15]

17. 404 and 410

Missing or removed pages · M2 · task 2.2

Layout: Centred: short message, search box, four specialty chips, contact link. 410 adds one line that the doctor asked to be removed. Above the fold: everything.

1. Search box plus popular specialties and a contact path; 410 explains the doctor was removed at their request and offers the specialty search. [S6 L4]

Patient

18. Sign up and login

OTP by email · M3 · task 3.6

Layout: Centred card: one email field and Continue; second step replaces it with one six-digit field, Open Gmail link, resend timer. Why we ask line under the field. Above the fold: the whole card.

1. One OTP field, inputmode numeric, autocomplete one-time-code, paste works, 16 px input; email keyboard, autocorrect off. `[F6 W4 F8]`
2. Open Gmail link; sign-in asked only at booking or upload, never to browse. `[F7 F9]`
3. Rate-limited state says when to retry; errors in text next to the field. `[F5 L4]`

19. Slot picker

Choose a time in the patient's timezone · M3 · task 3.2

Layout: Mini doctor card at the top, visit-type segmented control, week strip (swipe), slot grid of 48 px buttons in the patient zone with the zone named, doctor time in small text on the selected slot. Reason, age group and for whom appear after a slot is picked. Sticky Continue. Above the fold: card, visit type, this week's slots. Hook: the next available slot is pre-highlighted.

1. Patient zone named on every slot, doctor time secondary; Western digits forced; weekends shaded per the patient's country. `[P6 N1 P7]`
2. 48 px slot buttons; hold starts within 400 ms; calendar chevrons mirrored in RTL, clock icon not. `[W2 L2 R2]`
3. Ask visit reason, age group and for whom before the hold; visit type choice with best-for line. `[P8 C12]`
4. No slots this week: show next available week and a Request a time path, never an empty grid. `[L3 S6]`

20. Checkout

Pay for the first visit · M3 · task 3.2, 3.3

Layout: Single page, two blocks: Review (doctor, time in both zones, visit type, edit links) and Pay (three lines and total, then the card box with Apple Pay and Google Pay buttons above the card fields). Consent and cancellation checkboxes between the blocks. Hold timer in the sticky header. Confirmation is a separate page. Above the fold: review block and total. Hook: the hold timer.

1. Three lines sum to the total, nothing new; Review with edit links, then a distinct Confirmation. `[P1 C5 P2]`
2. Card fields in a bordered box with Secured by Stripe; Apple Pay and Google Pay first; expiry MM / YY; capture in full; keep details after an error. `[P3 P4 P12 P13]`
3. Cancellation terms and telehealth consent on this step; confidentiality line in plain Arabic. `[P5 P9 T12]`
4. Local currency, three decimals for KWD/OMR/BHD, Western digits, currency sign part of the number. `[P14 N2 N1]`
5. Backup phone and country; no re-asking of data from the slot picker. `[F10 F2]`
6. Hold timer visible; paying state shows steps, never a blank spinner. `[L1 T11]`

21. Booking confirmation

Show what was booked · M3 · task 3.3

Layout: Green check and one line, then the appointment card (time in both zones, join link), Add to calendar, then a three-item prepare checklist with Test your device, then Share records prompt, then receipt link. Above the fold: card and calendar button. Hook: Test your device.

1. Peak-end: both times with zones, join link, add to calendar, what to prepare, how to change, expected lead time. [P10 P6]
2. One-tap device check link; not-for-emergencies line with local number. [G4 H14]
3. Share records prompt explains AI consent and who will see them. [T3 T7]

22. Bookings list

See past and upcoming visits · M3 · task 3.6

Layout: Tabs Upcoming and Past. Each row: doctor photo, date and time in the patient zone, status chip, Join or View. Empty state with Find a doctor. Above the fold: next visit as a larger card, then the list.

1. Empty state: what it is, why empty, Find a doctor action; view is the primary action. [L3 T1]
2. Times in the patient zone; cancel uses outcome-named buttons (Cancel booking / Keep booking). [P6 B10]

23. Receipts

Itemised receipts · M3 · task 3.6

Layout: List of rows (date, doctor, total, status). Detail: total large, the three lines, refund entries below, download PDF. Above the fold: the list.

1. Total prominent, three lines, last 4 digits, both times, refund entries; written estimate retained; local currency. [E3 P15 P14]

24. Records upload

Upload files, get a quote · M3 · task 3.7

Layout: Dropzone with a camera button first on phone. Below it the file list with per-file status. Then the quote box (files, pages, total). Then the two consent checkboxes (privacy, AI processing with the where-stored line) and Pay. Above the fold: dropzone and file list. Hook: the quote updates as files land.

1. Separate AI-processing consent checkbox with a where your data is stored line; UAE residency to be settled (Q3). [T3 Q3]
2. Phone photos first (camera button), formats and size visible, per-file success or specific rejection reason. [T10 F8 L4]
3. Quote shown as one all-in number before consent; nothing processes before payment (as planned). [C5 P1]

25. Records processing

Show progress · M3 · task 3.8

Layout: One card: overall status line, then a step list with the current step live (page 4 of 12), cost so far, estimate, Cancel. Below: what you will get, as three lines. Above the fold: everything. Hook: the step list moving.

1. Live step list with page counts and cost so far; estimate and cancel when over 10 s; no fake percentage. [T11 L1]
2. Failed state names the reason and the fix (re-upload page 4). [L3 L4]

26. Summary and timeline

Read the one page summary · M3 · task 3.9, 3.10

Layout: AI-generated label and reviewed state at the top, Talk to a human beside it. Tabs Overview, Timeline, Flags. Overview: five blocks (diagnoses, procedures, medications, allergies, results), each item with a source-page link opening a side viewer (sheet on phone). Timeline is vertical on phone. Above the fold: label and first two blocks. Hook: tap any item to see the source page.

1. AI-generated label at the top, Reviewed by Dr X or Not yet reviewed, Talk to a human; not-a-diagnosis in the body. [T4 T5 Q6]
2. Release immediately with a framing line; every statement links to its source page; explain each number; something missing? link. [T2 T6 X1]
3. Timeline numerals LTR with mirrored axis in Arabic; download is a primary action. [R11 T1]

27. Sharing and consent

Share with one doctor · M3 · task 3.10

Layout: List of shares as rows (doctor, expires on, Revoke). Share with a doctor button opens a sheet listing booked doctors and an expiry picker. Who can see this panel pinned at the top. Above the fold: panel and first share.

1. Who can see this panel, expiry visible, revoke in one tap; plain confidentiality line. [T7 T12]
2. No shares: explain what sharing does and offer Share with your booked doctor. [L3]

28. Settings

Language, timezone, delete account · M3 · task 3.6

Layout: Grouped list: Language, Timezone, Calendar (Gregorian, Hijri on), Notifications, then a Danger section with Delete uploads and Delete account, each explaining retention. Above the fold: first two groups.

1. Delete explains what is removed and what retention keeps; confirmation buttons name the outcome. [T8 B10]
2. Timezone from a GCC-first list; Gregorian with optional Hijri; language in its own script. [P6 N3 H10]

Professional

29. Profile builder

Claim, onboard, edit, see the card live · M2 · task 2.4, 2.5

Layout: Phone: form sections stacked with a sticky mini card at the top that updates live; tap it to see the full card and flip. Desktop: form left, full card right, pinned. Section order: photo, names, specialty and city, languages, visit types and fees, training and credentials, bio, tags, availability link, publish checklist. Progress bar under the header starts part-filled. Above the fold: mini card, photo step. Hook: the card changing as you type.

1. Opens pre-filled from registry; required = NPI, licence, board, specialty, photo; checklist starts partly filled with steps remaining. `[B1 B2 B3]`
2. Live card is the reward; photo step says profiles with photos get about 2x views. `[B4 B5 C3]`
3. Photo guidance: white coat, plain background, no group or stock; reject obvious stock. `[C4 G2]`
4. Structured fields: years in practice, board and status (board eligible is text, not badge), training, languages with dialect, Accepting (off until availability set). `[B6 B7 S16]`
5. Labels above, chunks of 4 to 6, validate on blur, specific errors; Arabic fields budget 30% more width. `[F1 F3 F4 F5 R8]`
6. Arabic preview aligns English bio by its language; NPI and phone stay LTR. `[R4 R3]`

30. Availability editor

Weekly rules, exceptions, timezone · M2 · task 2.6

Layout: Weekly grid (seven columns, swipeable on phone) with tap-to-add rules, exceptions list below, slot length and timezone selectors above. Preview as Riyadh viewer toggle shows the patient's view. Above the fold: the week grid. Hook: the preview toggle.

1. Weekend shading follows the preview country; Riyadh preview shows zone and no-DST note; Gregorian. `[P7 P6 N3]`
2. Changes apply within 400 ms with undo, no confirmation dialogs. `[L2 B10]`
3. Saving the first rule flips Accepting new patients on, with a one-line notice. `[B6]`

31. Practice ID card

View and download the card · M2 · task 2.7

Layout: Card shown large, flip button, language toggle, Download PNG for each side. Unverified watermark until approved. Above the fold: the card.

1. Verification tick and logo never mirrored; Arabic +2 pt, letter-spacing 0, real weights. `[R2 R6 R5]`
2. Badge text: Credentials verified on [date]; unverified watermark until approved. `[C8 A2]`

32. Introduction inbox

See bookings and requests · M2 (placeholder), M3 (real) · task 2.9, 3.3

Layout: Rows: patient first name, date and time in the doctor zone, status, Join. Detail drawer: both times, join link, shared summary link with expiry. Above the fold: next visit large, then the list.

1. Empty state explains when requests appear; detail answers what, when (both zones), how to join. `[L3 E2]`
2. Shared summary link shows expiry and that the patient can revoke. `[T7]`

33. Membership and billing

Subscribe and manage · M3 · task 3.5

Layout: Current tier card (name, price, renewal), then Manage billing (Stripe portal), then the tier table for upgrades. Past due shows a gold banner at the top. Above the fold: tier card.

1. Tier, all-in price, renewal date; past-due state says the grace period and what stops. [B8 C5]
2. Cancel via Stripe portal; confirmation names the outcome. [B10]

34. Featured banner

Buy the carousel slot · January · task 3.5

Layout: Hidden until January. When shown: price, dates picker, preview of the home carousel slot with the Sponsored label visible in the preview.

1. Any purchased slot renders with a Sponsored label at the reading-start corner, shaded; same attributes as organic cards. [A4]

35. Payouts setup

Connect Stripe Express · M3 · task 3.1

Layout: One card: the doctor's practice card small, a sentence on how payouts work, three-step status list (identity, bank, enabled), the Stripe button. Restricted state lists what Stripe needs. Above the fold: everything.

1. Show the claimed card and expected earnings before the Stripe link; state the steps and that they can resume. [B5 B3 F3]
2. Restricted state names what Stripe needs. [L3]

36. Shared summaries

Read what patients shared · M3 · task 3.10

Layout: Rows: patient first name, shared on, expires on, Open. Viewer reuses the patient summary screen with the AI label and reviewed state. Above the fold: the list.

1. AI label and reviewed state carry through; expiry shown. [T4 T7]

Admin

37. Login with TOTP

Secure admin access · M2 · task 2.8

Layout: Centred card, three sequential steps replacing each other: email, code, authenticator code. No branding beyond the wordmark. Above the fold: the card.

1. Single-field codes, paste works, errors in text; refused state gives no hint which factor failed. [F6 W4 F5]

38. Credential review queue

Approve or reject claims · M2 · task 2.8

Layout: Left: queue list with age and status. Right: detail with the registry data and the doctor's entries side by side, documents as thumbnails opening in a viewer, a verification checklist (licence, board, sanctions, photo) each with source and date fields, then Approve or Reject with reason. Above the fold: the checklist.

1. Checklist per claim: licence (state board), board (ABMS), sanctions (FSMB/NPDB); store source, date, reviewer; board eligible never becomes the badge. `[A1 A2 B7]`
2. Photo check (white coat, no stock); approve/reject with outcome-named buttons; reject requires a reason the doctor sees. `[C4 B10]`

39. Listings

Manage professionals · M2 · task 2.8

Layout: Table with status filter chips above (Unclaimed, Claimed, Verified, Rejected, Removed), columns name, specialty, city, verified on, next re-verification; row click opens an edit drawer.

1. Verified-on date and next re-verification column; unclaimed clearly distinct. `[A2 A5 U1]`

40. Specialty list

Edit the 104 groups · M2 · task 2.8

Layout: Table: Arabic name, English name, slug, codes, synonyms; inline edit; Add row at the top.

1. Arabic and English names plus synonym and transliteration lists for search. `[S16 S2]`

41. Removal requests

Handle this is me / remove me · M2 · task 2.8

Layout: Table with SLA timer column (colour at 36 h), detail drawer with the request, matching profile, Confirm removal or Refuse with reason.

1. SLA timer visible; confirmed state triggers 410 and audit entry. `[A3 U4]`

42. Members and subscriptions

See memberships · M3 · task 3.5, 3.11

Layout: Table defaulting to Active and Past due; filter chips for the rest; Stripe link per row.

1. Table defaults to active and past due; the rest one level down. `[A5]`

43. Payments, refunds, payouts

Money view and refund action · M3 · task 3.4, 3.11

Layout: Three tabs. Payments table with status; Refund button opens a drawer with the amount, the transfer reversal note, and a confirm that names the outcome. Export CSV top right.

1. Refund (not void) with outcome-named confirmation; export mirrors receipt lines. `[P13 B10 E3]`

44. Banners

Manage featured slots · January · task 3.5

Layout: Hidden until January. Table of banners with label, dates, status; preview link.

1. Each banner row shows its label and dates; no unlabelled placements possible. `[A4]`

45. AI jobs and costs

Watch the records pipeline · M3 · task 3.11

Layout: Daily spend tile row at the top (today, this week, alert threshold), then the jobs table with status and Retry; LTR numerals in charts.

1. Charts with LTR numerals; empty state explains what a job is. [R11 L3]

46. Outreach and invites

Track invite tokens · M2 · task 2.3, 4.5

Layout: Funnel tiles (sent, opened, claimed, expired) then the table with filters; send actions stay disabled until counsel clears sending.

1. Track opened and claimed; filters one level down. [U6 A5]

47. Audit log

Who did what · M2 · task 2.8

Layout: Table with actor, action, target, old and new (expandable), time; filters by actor and action; no edit controls anywhere.

1. Actor, source and reviewer on verification events; append only. [A1 B10]

Supporting

48. Brand page /brand

Tokens and components · M1 · task 1.2

Layout: Left contents rail, sections: colours, type (both scripts side by side with the Arabic scale), spacing, icons with a mirror / do not mirror table, buttons, fields and errors, tags, skeletons, empty states, DoctorCard in every state, coach mark. Locale toggle at the top flips the whole page.

1. Tokens: Arabic type scale (16/18 px body, +2 pt beside Latin, line height 1.5), letter-spacing 0 for ar, font-synthesis none, 30% width budget; contrast 4.5:1 checked for gold on sand; 24 and 48 px targets. [R5 R6 R8 W1 W2]
2. Icon mirroring list (mirror vs never); bdi and dir=ltr helpers; number formatter with nu-latn. [R2 R3 N1]
3. Form field, error, skeleton and empty-state components built once here. [F1 F5 L1 L3]

49. Email templates (8)

Transactional email · M2, M3 · task 2.9, 3.3, 3.4

Layout: One layout: wordmark, one-line headline, the key block (code, appointment card, or receipt lines), one button, plain-text details, footer with address. Width 600 px, single column, RTL variant per template.

1. Subject states the content; body answers what, when (both zones), how to join, how to change; receipt mirrors review; RTL with LTR codes, 16 px Arabic. [E1 E2 E3 E4]
2. Any AI-generated line in an email is labelled. [T4]

50. Loading and empty states

Every route · M1 onward · task 1.6, 1.7

Layout: Skeletons are shaped like the real rows and cards and use real labels; empty states are a short title, one line of why, one button; both live on the brand page and are reused, never redrawn per screen.

1. Nielsen limits (1 s, 10 s), 400 ms optimistic response, three-part empty states, labour-illusion step lists, specific errors. [L1 L2 L3 L4 T11]

Sources

133 distinct sources. Every URL was fetched during the research on 19 Oct 2026 unless the lane report noted otherwise; a few paywalled or JavaScript-only pages were read through summaries, and those are named as such in the source column.

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